

Neurologic Deficit - Female (Adult)

Algorithm 1230 | Adult (18 years and older) | Gender at birth: Female

Definition

- Safety-first assessment of a new or sudden focal neurologic deficit in a child or adult. This protocol does not diagnose stroke or a stroke mimic; emergency assessment is required.
- Adult patient.
- Female.

Initial assessment

1. [DURATION] What is the exact symptom-onset time and the last-known-well time? If symptoms were noticed on waking, when was the person last known normal before sleep?
2. [OPEN_TEXT] Check for sudden face droop, one-sided arm or leg weakness or numbness, speech or understanding difficulty, new loss or double vision, severe dizziness, loss of balance or coordination, inability to walk, or a sudden severe headache.
3. [YES_NO] Is the person difficult to wake, not breathing normally, or having a seizure now?
4. [OPEN_TEXT] For a child, record age and any seizure, recent head injury, fever, possible ingestion, or known neurologic condition. These details must not delay the emergency call.
5. [OPEN_TEXT] Does the person have diabetes, and is a glucose reading immediately available? Checking glucose must not delay calling 999.

Triage and redirect questions

1. Redirect → Fainting

Was there a brief faint with full recovery and no ongoing weakness, speech, vision, balance, or facial change?

A fully recovered brief loss of consciousness without a continuing neurologic deficit belongs in the Fainting pathway.

1. Disposition level 100

Is there any sudden face droop, one-sided arm or leg weakness or numbness, abnormal speech or understanding, new loss or double vision, severe dizziness, loss of balance or coordination, inability to walk, sudden severe headache, reduced consciousness, or seizure - including in a child and even if the symptoms improved or stopped?

FAST signs and sudden vision, balance, coordination, walking, or severe-headache symptoms can indicate stroke. Hypoglycaemia, seizure, migraine, head injury, infection, and other conditions may mimic stroke, particularly in children, but telephone triage cannot safely distinguish them and must not delay emergency assessment.

Advice

Emergency stroke precautions

- Call Qatar 999 for an ambulance now. Record the exact onset and last-known-well times, including the time last known normal before sleep for wake-up symptoms. Keep the person safe and at rest; do not allow self-driving and do not give food, drink, or medicines because swallowing may be unsafe. If unconscious but breathing normally, place in the recovery position if safe; if not breathing normally, follow the 999 call-handler's resuscitation instructions. A glucose check or possible alternative diagnosis must not delay the call. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: symptoms worsen; loss of consciousness.

First aid

- Call Qatar 999 for an ambulance now. Record the exact onset and last-known-well times, including the time last known normal before sleep for wake-up symptoms. Keep the person safe and at rest; do not allow self-driving and do not give food, drink, or medicines because swallowing may be unsafe. If unconscious but breathing normally, place in the recovery position if safe; if not breathing normally, follow the 999 call-handler's resuscitation instructions. A glucose check or possible alternative diagnosis must not delay the call. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Stroke - Symptoms", <https://www.nhs.uk/conditions/stroke/symptoms/> (page last reviewed 12 September 2024)
- US CDC, "Signs and Symptoms of Stroke", <https://www.cdc.gov/stroke/signs-symptoms/> (accessed 25 July 2026)
- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (999 ambulance access; accessed 25 July 2026)

Provenance

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